

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0001
Name: Rajesh Kumar
Date of Birth: July 12, 1975 (50 years old)
Gender: Male
Blood Type: B+
MRN: 100001
Insurance: Ayushman Bharat - PM-JAY
VISIT DATE: September 4, 2025
Provider: Dr. Ananya Joshi, MD
Visit Type: Comprehensive Follow-up
CHIEF COMPLAINT

"Recurrent low-grade fever and joint pains for 2 weeks"
ALLERGIES

No known drug allergies
HISTORY OF PRESENT ILLNESS

Mr. Kumar is a 50-year-old male with a history of type 2 diabetes and hypertension who presents with low-grade evening fever, generalized body ache, and pain in small joints of hands and feet for the past 2 weeks. He reports mild headache and fatigue. No rash, no bleeding, and no breathlessness. He recently returned from a work trip to Delhi during monsoon season.
PAST MEDICAL HISTORY

1. Type 2 Diabetes Mellitus - diagnosed 2018
 2. Hypertension - diagnosed 2016
 3. Dengue fever - treated 2022
 4. Dyslipidemia - diagnosed 2021
- MEDICATIONS

1. Metformin 1000mg PO BID
 2. Telmisartan 40mg PO daily
 3. Amlodipine 5mg PO at night
 4. Atorvastatin 20mg PO daily
- SOCIAL HISTORY

- Occupation: Bank manager (mostly desk work)
 - Tobacco: Never smoked
 - Alcohol: Occasional, 1-2 pegs/month
 - Exercise: Walks 20-30 minutes, 4 days a week
 - Diet: Traditional Indian vegetarian diet, high in carbohydrates, moderate oil use
- FAMILY HISTORY

Father: Type 2 DM, hypertension
Mother: Osteoarthritis
Brother: Hypertension
Children: 2, healthy
REVIEW OF SYSTEMS

Constitutional: Positive for fever and fatigue, no weight loss
Musculoskeletal: Joint pains in wrists, ankles, and small joints of hands
Respiratory: No cough or shortness of breath
GI: Normal appetite, no vomiting or loose stools
Neurological: No focal weakness or seizures
PHYSICAL EXAMINATION

Vital Signs:

- BP: 136/84 mmHg
- HR: 88 bpm, regular
- RR: 18/min
- Temp: 99.5°F
- O2 Sat: 99% on room air
- BMI: 27.4 kg/m²

General: Alert, mildly uncomfortable due to joint pains

HEENT: No icterus, no pallor

Cardiovascular: S1S2 heard, no murmurs

Pulmonary: Clear to auscultation bilaterally

Abdomen: Soft, non-tender, no hepatosplenomegaly

Extremities: Mild tenderness in small joints of hands and ankles, no obvious swelling

LABORATORY RESULTS (Today)

HbA1c: 7.8%

Fasting Glucose: 152 mg/dL

Platelet count: 1.8 lakh/ μ L (normal)

WBC: 7,800 /mm³

Hemoglobin: 13.4 g/dL

ESR: 35 mm/hr (elevated)

CRP: mildly elevated

Dengue NS1: Negative

Dengue IgM: Negative

Chikungunya IgM: Positive

Liver and renal function tests: Within normal limits

ASSESSMENT & PLAN

1. ACUTE CHIKUNGUNYA FEVER WITH ARTHRALGIA

- Symptomatic treatment with paracetamol and adequate hydration
- Avoid NSAIDs due to diabetes and blood pressure
- Advise rest and limb elevation when possible

2. TYPE 2 DIABETES MELLITUS

- Continue Metformin
- Reinforce dietary control and regular blood sugar monitoring

3. HYPERTENSION

- Continue Telmisartan and Amlodipine
- Monitor BP at home twice weekly

FOLLOW-UP

- - Review in 1 week or earlier if high-grade fever, breathlessness, or bleeding develops
 - Return immediately if joint pains worsen severely or rash/bleeding appears

PATIENT EDUCATION

Patient counselled about mosquito-borne illnesses common in India, importance of mosquito control measures (nets, repellents, avoiding stagnant water), adherence to medicines, and monitoring of blood sugars and blood pressure.

Patient verbalized understanding and agreed to follow-up.

Electronically signed by: Ananya Joshi, MD

Date/Time: September 4, 2025, 11:30

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0002
Name: Priya Sharma
Date of Birth: February 2, 1988 (37 years old)
Gender: Female
Blood Type: O+
MRN: 100002
Insurance: Employee State Insurance (ESI)
VISIT DATE: September 5, 2025
Provider: Dr. Rohan Mehta, MD
Visit Type: New Consultation
CHIEF COMPLAINT

"Breathlessness on exertion and long-standing cough"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Ms. Sharma is a 37-year-old woman from a semi-urban area who presents with progressive breathlessness on exertion and chronic cough with sputum for the last 8 months. She reports wheezing episodes at night and early morning. No hemoptysis or weight loss. She has a history of using biomass (chulha) for cooking for many years before LPG connection was provided.

PAST MEDICAL HISTORY

1. Childhood asthma - intermittent
2. Recurrent lower respiratory infections
3. Iron deficiency anemia - diagnosed 2021

MEDICATIONS

1. Budesonide/Formoterol inhaler 2 puffs BD (recently started by local physician)
2. Oral iron supplements off and on

SOCIAL HISTORY

- Occupation: School teacher
- Tobacco: Never smoked; husband smokes occasionally at home
- Alcohol: None
- Exercise: Limited due to breathlessness
- Environmental exposure: Biomass fuel exposure for ~12 years, now on LPG

FAMILY HISTORY

Father: Hypertension
Mother: Type 2 DM
Sister: Hypothyroidism

REVIEW OF SYSTEMS

Respiratory: Chronic cough, wheeze, exertional dyspnea
Constitutional: Fatigue, no fever, mild weight loss
Cardiovascular: No chest pain or palpitations
GI: Normal appetite

PHYSICAL EXAMINATION

Vital Signs:

- BP: 122/78 mmHg
- HR: 96 bpm
- RR: 20/min
- Temp: 98.4°F
- O2 Sat: 96% on room air

- BMI: 22.1 kg/m²

General: Mild respiratory distress on walking into room

HEENT: No clubbing, no cyanosis

Cardiovascular: S1S2, no murmurs

Pulmonary: Bilateral diffuse wheeze, prolonged expiratory phase

Abdomen: Soft, non-tender

Extremities: No edema

LABORATORY AND IMAGING RESULTS

Hemoglobin: 10.2 g/dL (low)

WBC: 7,200 /mm³

Chest X-ray: Hyperinflated lung fields, no focal consolidation

Spirometry: Moderate obstructive pattern with partial reversibility

ASSESSMENT & PLAN

1. CHRONIC OBSTRUCTIVE AIRWAY DISEASE (probable COPD with asthma overlap) related to biomass exposure

- Continue inhaled bronchodilator plus steroid

- Add short-acting inhaler as rescue

- Teach inhaler technique and peak-flow monitoring

- Vaccination advice: annual influenza and pneumococcal as per national guidelines

2. IRON DEFICIENCY ANEMIA

- Start oral iron therapy with folic acid

- Dietary counselling for iron-rich Indian foods (green leafy vegetables, jaggery, pulses)

FOLLOW-UP

- Follow up in 4 weeks with repeat clinical assessment and review of inhaler use

- Earlier review if increased breathlessness, fever, or chest pain

PATIENT EDUCATION

Discussed importance of avoiding smoke exposure (including second-hand tobacco), regular use of inhalers, pulmonary rehabilitation exercises, and nutrition. Patient and spouse advised to keep home well-ventilated and avoid burning wood or waste inside the house.

Electronically signed by: Rohan Mehta, MD

Date/Time: September 5, 2025, 10:15

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0003
Name: Ayesha Khan
Date of Birth: November 20, 1996 (28 years old)
Gender: Female
Blood Type: A+
MRN: 100003
Insurance: Self-pay
VISIT DATE: September 6, 2025
Provider: Dr. Neeraj Kulkarni, MD
Visit Type: Follow-up
CHIEF COMPLAINT

"Tingling and burning sensation in feet for 3 months"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Ms. Khan is a 28-year-old woman, strictly vegetarian, who presents with progressive tingling, numbness, and burning sensation in both feet for the last 3 months, worse at night. She reports fatigue and occasional dizziness. No weakness of limbs or visual disturbances. No history of diabetes. She frequently skips breakfast and consumes mostly tea and bread.

PAST MEDICAL HISTORY

1. Vitamin D deficiency - treated 2022
2. Dyspepsia on and off

MEDICATIONS

1. Occasional antacid syrup

SOCIAL HISTORY

- Occupation: Call center executive, works night shifts
- Tobacco: None
- Alcohol: None
- Diet: Strict vegetarian, high tea intake, low fruits and vegetables
- Sleep: Irregular due to night shifts

FAMILY HISTORY

Father: Type 2 DM
Mother: Hypothyroidism
Brother: Healthy

REVIEW OF SYSTEMS

Neurological: Tingling and burning in feet, no focal weakness

Constitutional: Fatigue, no fever

GI: Occasional bloating, no vomiting

PHYSICAL EXAMINATION

Vital Signs:

- BP: 110/70 mmHg
- HR: 82 bpm
- RR: 16/min
- Temp: 98.2°F
- O2 Sat: 99% on room air
- BMI: 20.4 kg/m²

General: Thin built, cooperative

Neurological: Decreased vibration and position sense in feet, ankle reflexes reduced

Cardiovascular/Respiratory/Abdomen: Within normal limits

LABORATORY RESULTS (Today)

Hemoglobin: 9.8 g/dL (low)

MCV: 108 fL (macrocytosis)

Vitamin B12 level: 110 pg/mL (low)

Folate level: Normal

Fasting glucose: 92 mg/dL

Thyroid profile: Normal

ASSESSMENT & PLAN

1. VITAMIN B12 DEFICIENCY WITH PERIPHERAL NEUROPATHY (likely dietary)

- Start intramuscular vitamin B12 injections as per protocol followed in hospital
- Plan to switch to oral B12 after loading doses

2. NUTRITIONAL ANEMIA (macrocytic)

- Counsel on balanced vegetarian diet including dairy products, fortified cereals, and, if acceptable, vitamin supplements

3. NIGHT-SHIFT RELATED LIFESTYLE ISSUES

- Sleep hygiene counselling, limit excess tea/coffee, regular meals

FOLLOW-UP

- Follow up in 2 weeks after B12 injections started, then every 3 months

PATIENT EDUCATION

Explained common nutritional deficiencies seen in Indian vegetarian diets and importance of regular meals, B12 supplementation, and follow-up to prevent progression of neuropathy.

Electronically signed by: Neeraj Kulkarni, MD

Date/Time: September 6, 2025, 09:45

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0004
Name: Suresh Patel
Date of Birth: May 5, 1963 (62 years old)
Gender: Male
Blood Type: AB+
MRN: 100004
Insurance: CGHS
VISIT DATE: September 7, 2025
Provider: Dr. Kavita Desai, MD
Visit Type: Comprehensive Follow-up
CHIEF COMPLAINT

"Swelling of feet and reduced urine output"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Mr. Patel is a 62-year-old man with long-standing diabetes and hypertension who presents with gradually increasing swelling of both legs, facial puffiness in the morning, and reduced urine output for the last 3 weeks. He complains of easy fatigability and decreased appetite. No breathlessness at rest, no chest pain.

PAST MEDICAL HISTORY

1. Type 2 Diabetes Mellitus - diagnosed 2005
2. Hypertension - diagnosed 2008
3. Diabetic retinopathy - diagnosed 2020

MEDICATIONS

1. Metformin 1000mg PO BID
2. Glimepiride 2mg PO daily
3. Losartan 50mg PO BID
4. Amlodipine 5mg PO daily

SOCIAL HISTORY

- Occupation: Retired government clerk
- Tobacco: Chews tobacco (mishri) 3-4 times daily
- Alcohol: None
- Diet: Mixed, high salt intake, enjoys pickles and fried snacks

FAMILY HISTORY

Father: Died of stroke at 70
Mother: Type 2 DM
Brother: Hypertension

REVIEW OF SYSTEMS

Renal: Reduced urine output, frothy urine
Cardiovascular: No chest pain, occasional exertional dyspnea
GI: Poor appetite, occasional nausea

PHYSICAL EXAMINATION

Vital Signs:

- BP: 150/92 mmHg
- HR: 84 bpm
- RR: 18/min
- Temp: 98.6°F
- O2 Sat: 97% on room air

- BMI: 26.8 kg/m²

General: Mild pallor, facial puffiness

Cardiovascular: S1S2, no murmurs

Pulmonary: Clear

Abdomen: Soft, no organomegaly

Extremities: Bilateral pitting pedal edema up to mid-shin

LABORATORY RESULTS (Today)

Serum Creatinine: 2.4 mg/dL (elevated)

eGFR: 30 mL/min/1.73m² (reduced)

Blood Urea: 78 mg/dL (elevated)

Urine routine: Protein 3+, few RBCs

HbA1c: 8.6%

Electrolytes: Within normal limits

Ultrasound abdomen: Bilaterally small, echogenic kidneys

ASSESSMENT & PLAN

1. DIABETIC CHRONIC KIDNEY DISEASE - STAGE 3B

- Start ACEi/ARB optimization as tolerated, adjust antihypertensives

- Restrict salt and protein as per nephrology diet advice

- Refer to nephrologist for detailed CKD management and renal replacement counselling

2. UNCONTROLLED TYPE 2 DIABETES

- Review current regimen; consider adding insulin based on nephrology opinion

3. HYPERTENSION

- Tight BP control with combination therapy

FOLLOW-UP

- Nephrology visit within 1 week

- Internal medicine review in 2 weeks with latest renal function tests

PATIENT EDUCATION

Counselled patient and family about chronic kidney disease due to long-standing uncontrolled diabetes, importance of strict sugar and BP control, low-salt diet, avoiding painkillers, and regular follow-up to delay progression.

Electronically signed by: Kavita Desai, MD

Date/Time: September 7, 2025, 12:10

PATIENT MEDICAL RECORD

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BHARAT CARE HOSPITAL
Department of Internal Medicine

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PATIENT INFORMATION

Patient ID: IND-2025-0005
Name: Lakshmi Narayan
Date of Birth: January 10, 1955 (70 years old)
Gender: Female
Blood Type: B-
MRN: 100005
Insurance: Ayushman Bharat - PM-JAY
VISIT DATE: September 8, 2025
Provider: Dr. Arjun Nair, MD
Visit Type: Follow-up
CHIEF COMPLAINT

"Joint pains in knees and hands, worse in the morning"
ALLERGIES

Diclofenac - gastric upset
HISTORY OF PRESENT ILLNESS

Mrs. Narayan is a 70-year-old woman who presents with chronic pain and stiffness in both knees and small joints of hands. Morning stiffness lasts about 30 minutes. Pain increases with walking and climbing stairs. No history of trauma. She lives in a village and often sits on the floor for household work.

PAST MEDICAL HISTORY

-
1. Osteoarthritis of knees - diagnosed 2018
 2. Hypothyroidism - diagnosed 2015
 3. Vitamin D deficiency - diagnosed 2020

MEDICATIONS

-
1. Levothyroxine 75 mcg PO daily
 2. Calcium and vitamin D supplements
 3. Occasional paracetamol for pain

SOCIAL HISTORY

-
- Occupation: Homemaker
 - Tobacco: None
 - Alcohol: None
 - Diet: Vegetarian, adequate dairy intake

FAMILY HISTORY

Husband: Hypertension
Son: Healthy
Daughter: Rheumatoid arthritis

REVIEW OF SYSTEMS

Musculoskeletal: Chronic knee pain, hand joint pain
Constitutional: No fever, stable weight
Endocrine: On treatment for hypothyroidism

PHYSICAL EXAMINATION

Vital Signs:

- BP: 128/80 mmHg
- HR: 76 bpm
- RR: 16/min
- Temp: 98.4°F
- BMI: 25.0 kg/m²

General: Comfortable, ambulates slowly

Musculoskeletal: Crepitus in both knees, mild bony swelling, no warmth; mild tenderness in PIP joints of both hands

LABORATORY AND IMAGING RESULTS

TSH: 3.0 mIU/L (controlled)

Vitamin D: 28 ng/mL (low-normal)

X-ray knees: Osteoarthritic changes with joint space narrowing

Rheumatoid factor: Negative

ASSESSMENT & PLAN

1. OSTEOARTHRITIS KNEES WITH HAND JOINT INVOLVEMENT

- Continue paracetamol as needed within safe limits
- Start physiotherapy focusing on quadriceps strengthening
- Advise avoidance of squatting and sitting cross-legged

2. HYPOTHYROIDISM - CONTROLLED

- Continue current Levothyroxine dose

FOLLOW-UP

- Review in 3 months or earlier if pain worsens

PATIENT EDUCATION

Explained degenerative joint disease common in elderly Indian women, posture modification, weight management, and role of regular exercises. Discussed red-flag symptoms requiring early review (sudden swelling, redness, fever).

Electronically signed by: Arjun Nair, MD

Date/Time: September 8, 2025, 15:20

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0006
Name: Manoj Singh
Date of Birth: September 9, 1990 (35 years old)
Gender: Male
Blood Type: O+
MRN: 100006
Insurance: Private insurance
VISIT DATE: September 9, 2025
Provider: Dr. Shweta Rao, MD
Visit Type: New Consultation
CHIEF COMPLAINT

"High fever with chills and headache for 4 days"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Mr. Singh is a 35-year-old man who presents with high-grade intermittent fever with chills, severe headache, and body ache for 4 days. He reports nausea and poor appetite. No cough or urinary complaints. He lives in a suburban area with recent reports of dengue and malaria cases.

PAST MEDICAL HISTORY

1. No known chronic illnesses

MEDICATIONS

1. Took over-the-counter paracetamol and ibuprofen intermittently

SOCIAL HISTORY

- Occupation: Sales executive, travels frequently
- Tobacco: Smokes 5-6 cigarettes/day
- Alcohol: Social drinking on weekends
- Diet: Mixed

FAMILY HISTORY

Non-contributory

REVIEW OF SYSTEMS

Constitutional: High fever, chills, myalgia

GI: Nausea, no vomiting or loose stools

Neurological: Severe frontal headache

PHYSICAL EXAMINATION

Vital Signs:

- BP: 118/76 mmHg
- HR: 104 bpm
- RR: 18/min
- Temp: 102.4°F
- O2 Sat: 98% on room air

General: Ill-looking but oriented

HEENT: No neck rigidity, no icterus

Cardiovascular/Respiratory: Normal

Abdomen: Mild right upper quadrant tenderness, no organomegaly

LABORATORY RESULTS (Today)

Hemoglobin: 13.0 g/dL

WBC: 4,200 /mm³

Platelets: 90,000 / μ L (low)
Liver enzymes: Mildly elevated
NS1 antigen: Positive
Malaria rapid test: Negative

ASSESSMENT & PLAN

1. DENGUE FEVER WITHOUT WARNING SIGNS (currently)

- Advise adequate oral fluids and paracetamol for fever
- Avoid NSAIDs such as ibuprofen
- Daily monitoring of vitals and platelet count for next few days

2. SMOKING

- Brief counselling on tobacco cessation and available support

FOLLOW-UP

- Review daily for 3 days or sooner if warning signs: persistent vomiting, severe abdominal pain, bleeding, breathlessness, lethargy

PATIENT EDUCATION

Explained nature of dengue, need for mosquito control and prevention, importance of avoiding self-medication with painkillers, and recognising warning signs requiring immediate hospital visit.

Electronically signed by: Shweta Rao, MD

Date/Time: September 9, 2025, 13:00

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0007
Name: Sunita Devi
Date of Birth: August 18, 1970 (55 years old)
Gender: Female
Blood Type: A-
MRN: 100007
Insurance: Government health scheme
VISIT DATE: September 10, 2025
Provider: Dr. Vivek Menon, MD
Visit Type: Follow-up
CHIEF COMPLAINT

"Chronic cough with weight loss"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Mrs. Devi is a 55-year-old woman from a rural area who presents with chronic cough for 3 months, mostly dry, associated with loss of about 6 kg weight and night sweats. She reports occasional low-grade fever. She had contact with a neighbour recently treated for pulmonary tuberculosis.

PAST MEDICAL HISTORY

1. No known chronic illnesses

MEDICATIONS

1. Occasional cough syrup from local chemist

SOCIAL HISTORY

- Occupation: Agricultural labourer
- Tobacco: Chews pan with tobacco regularly
- Alcohol: None
- Living conditions: Crowded house with poor ventilation

FAMILY HISTORY

No family history of tuberculosis

REVIEW OF SYSTEMS

Respiratory: Chronic cough, occasional breathlessness

Constitutional: Weight loss, night sweats, fever

GI: Normal appetite but eats less due to weakness

PHYSICAL EXAMINATION

Vital Signs:

- BP: 116/74 mmHg
- HR: 92 bpm
- RR: 20/min
- Temp: 99.2°F
- O2 Sat: 97% on room air

General: Thin, mildly pale

Pulmonary: Crackles in right upper lung field

Cardiovascular/Abdomen: Unremarkable

LABORATORY AND IMAGING RESULTS

Hemoglobin: 10.5 g/dL

Sputum CBNAAT: Mycobacterium tuberculosis detected, rifampicin sensitive

Chest X-ray: Right upper lobe infiltrates with cavitation

HIV test: Negative

ASSESSMENT & PLAN

1. PULMONARY TUBERCULOSIS - NEW, DRUG-SENSITIVE

- Start daily regimen as per National TB Elimination Programme (NTEP) protocol
- Notify case to local TB unit

2. UNDERNUTRITION

- Nutritional counselling with locally available high-protein foods (dal, eggs if acceptable, milk, peanuts)

FOLLOW-UP

- Monthly follow-up in DOTS centre and hospital review in 2 months

PATIENT EDUCATION

Counselled about adherence to full TB treatment course, cough hygiene, mask use, and importance of screening household contacts. Explained that TB is common but curable with complete therapy.

Electronically signed by: Vivek Menon, MD

Date/Time: September 10, 2025, 11:20

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0008
Name: Rohit Verma
Date of Birth: December 1, 2004 (20 years old)
Gender: Male
Blood Type: B+
MRN: 100008
Insurance: Student health cover
VISIT DATE: September 11, 2025
Provider: Dr. Meera Sinha, MD
Visit Type: New Consultation
CHIEF COMPLAINT

"Recurrent loose stools and abdominal cramps"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Mr. Verma is a 20-year-old college student who presents with intermittent loose stools, abdominal cramps, and occasional low-grade fever for 10 days. He reports eating roadside food frequently during college hours. No blood in stools. No recent travel outside city.

PAST MEDICAL HISTORY

1. No significant past illnesses

MEDICATIONS

1. Took over-the-counter antidiarrheal tablets for 2 days

SOCIAL HISTORY

- Occupation: College student
- Tobacco: Occasionally smokes with friends
- Alcohol: Occasional beer with peers
- Diet: Frequently consumes street food and spicy snacks

FAMILY HISTORY

Non-contributory

REVIEW OF SYSTEMS

GI: Intermittent crampy abdominal pain, loose stools 4-5 times/day

Constitutional: Mild fatigue, no significant weight loss

PHYSICAL EXAMINATION

Vital Signs:

- BP: 112/70 mmHg
- HR: 88 bpm
- RR: 16/min
- Temp: 99.0°F
- BMI: 21.3 kg/m²

General: Mild dehydration

Abdomen: Mild diffuse tenderness, no guarding, normal bowel sounds

LABORATORY RESULTS (Today)

Stool routine: No blood, no ova/cysts

Widal test: Negative

Complete blood count: Mild leukocytosis

CRP: Mildly elevated

ASSESSMENT & PLAN

1. ACUTE INFECTIVE GASTROENTERITIS - LIKELY FOODBORNE

- Oral rehydration solution and plenty of fluids
- Short course of appropriate oral antibiotics as per local guidelines
- Avoid street food, raw salads, and uncovered eatables

FOLLOW-UP

- Follow up in 3 days or earlier if high fever, blood in stools, or severe abdominal pain

PATIENT EDUCATION

Explained common food- and water-borne illnesses in Indian settings, safe food and water practices, hand hygiene, and importance of completing prescribed medicines.

Electronically signed by: Meera Sinha, MD

Date/Time: September 11, 2025, 14:05

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0009
Name: Gopal Reddy
Date of Birth: April 22, 1972 (53 years old)
Gender: Male
Blood Type: O-
MRN: 100009
Insurance: State government employee scheme
VISIT DATE: September 12, 2025
Provider: Dr. Nidhi Kapoor, MD
Visit Type: Follow-up
CHIEF COMPLAINT

"Chest discomfort on walking and climbing stairs"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Mr. Reddy is a 53-year-old man with hypertension and dyslipidemia who presents with exertional central chest heaviness radiating to left arm for the last 1 month. Symptoms subside with rest in 5-10 minutes. No rest pain. No syncope. Strong family history of early heart disease.

PAST MEDICAL HISTORY

1. Hypertension - diagnosed 2015
2. Dyslipidemia - diagnosed 2018
3. Overweight

MEDICATIONS

1. Telmisartan 40mg PO daily
2. Atorvastatin 20mg PO daily

SOCIAL HISTORY

- Occupation: Office administrator
- Tobacco: Smoked for 15 years, quit 3 years ago
- Alcohol: Occasional
- Diet: High in refined carbohydrates and oil, eats late dinners

FAMILY HISTORY

Father: Myocardial infarction at 54
Mother: Hypertension

REVIEW OF SYSTEMS

Cardiovascular: Exertional chest discomfort, no palpitations

Respiratory: No chronic cough

Endocrine: No history of diabetes

PHYSICAL EXAMINATION

Vital Signs:

- BP: 142/88 mmHg
- HR: 80 bpm
- RR: 18/min
- Temp: 98.4°F
- BMI: 29.0 kg/m²

General: Overweight male, no acute distress at rest

Cardiovascular: S1S2, no murmurs

Respiratory: Clear

LABORATORY AND INVESTIGATIONS

Fasting lipid profile:

Total cholesterol: 236 mg/dL

LDL: 150 mg/dL

HDL: 36 mg/dL

Triglycerides: 210 mg/dL

Resting ECG: Non-specific ST-T changes

Treadmill test: Positive for inducible ischemia

ASSESSMENT & PLAN

1. STABLE ANGINA – SUSPECTED CORONARY ARTERY DISEASE

- Start Aspirin 75mg daily
- Start beta-blocker and long-acting nitrate as per protocol
- Refer to cardiologist for further evaluation (possible angiography)

2. HYPERTENSION AND DYSLIPIDEMIA – SUBOPTIMALLY CONTROLLED

- Optimize antihypertensives
- Increase statin dose

FOLLOW-UP

- Cardiology appointment within 1 week
- Lifestyle modification counselling (diet, exercise, weight loss)

PATIENT EDUCATION

Discussed high burden of coronary artery disease in Indian population at younger ages, importance of regular exercise, weight reduction, avoiding tobacco, and strict control of BP and lipids.

Electronically signed by: Nidhi Kapoor, MD

Date/Time: September 12, 2025, 16:40

PATIENT MEDICAL RECORD

BHARAT CARE HOSPITAL
Department of Internal Medicine

PATIENT INFORMATION

Patient ID: IND-2025-0010
Name: Kavya Iyer
Date of Birth: September 30, 2010 (15 years old)
Gender: Female
Blood Type: A+
MRN: 100010
Insurance: School health programme
VISIT DATE: September 13, 2025
Provider: Dr. Sameer Gupta, MD
Visit Type: New Consultation
CHIEF COMPLAINT

"Easy tiredness and decreased concentration in school"

ALLERGIES

No known drug allergies

HISTORY OF PRESENT ILLNESS

Kavya is a 15-year-old school-going girl brought by her mother with complaints of easy fatigability, reduced concentration in studies, and shortness of breath on climbing stairs for the last 2 months. She has heavy menstrual periods lasting 7-8 days. No history of chronic illness or recent infection.

PAST MEDICAL HISTORY

1. No significant illnesses

MEDICATIONS

None

SOCIAL HISTORY

- Student, lives with parents
- Diet: Predominantly vegetarian, often skips breakfast, low intake of green leafy vegetables

FAMILY HISTORY

Mother: History of anemia

REVIEW OF SYSTEMS

Constitutional: Fatigue, decreased exercise tolerance
Cardiorespiratory: Mild exertional breathlessness, no chest pain
Gynecological: Heavy menstrual bleeding

PHYSICAL EXAMINATION

Vital Signs:

- BP: 104/66 mmHg
- HR: 96 bpm
- RR: 18/min
- Temp: 98.6°F
- BMI: 19.0 kg/m²

General: Pale, otherwise well

Cardiovascular: Tachycardia, no murmurs

Respiratory/Abdomen: Normal

LABORATORY RESULTS (Today)

Hemoglobin: 8.4 g/dL (low)
MCV: 72 fL (microcytic)
Serum ferritin: Low
Peripheral smear: Microcytic hypochromic anemia

ASSESSMENT & PLAN

1. IRON DEFICIENCY ANEMIA SECONDARY TO MENORRHAGIA AND NUTRITIONAL DEFICIT

- Start oral iron therapy with vitamin C
- Dietary counselling to include iron-rich foods (green leafy vegetables, jaggery, sprouts)
- Refer to gynecology for evaluation of menorrhagia

FOLLOW-UP

- Review in 4 weeks with repeat hemoglobin level

PATIENT EDUCATION

Educated patient and mother about high prevalence of anemia among Indian adolescent girls, importance of diet, compliance with iron therapy, and menstrual tracking.

Electronically signed by: Sameer Gupta, MD

Date/Time: September 13, 2025, 10:50